Patient Name: Finn Wade

DOB: 01/15/1974

Reason for referral: chronic diarrhea

Patient phone: 530-555-2044

Primary insurance: Medicare

Service area: outside service area

Patient Name: Finn Wade
DOB: 01/15/1974
Reason for referral: chronic diarrhea
Patient phone: 580-555-2047
Primary Insurance: Medicare
Service area: outside service area

GI FAST TRACK – Direct Access Colonoscopy Referral Form

Center for Digestive Health
Phone: 407-241-3205

Please complete form, return with demographics, copy of insurance card, and recent H&P to Secure Email: directaccess@cdhfl.com or dedicated Fax#: *407-219-4585*

PATIENT INFORMATION

Date_____

Patient Name: _____

DOB: _____ Patient Phone#: _____

Referring Provider: _____

Referring Office Phone: _____ Fax: _____

Insurance: _____Member ID: _____

Primary Care Provider (if different than referring): _____

REFERRING PROVIDER AUTHORIZATION

GI Provider Name: _____

Referral/Auth #: _____

GI FAST TRACK ELIGIBILITY (ALL MUST BE YES)

☐ Age 45–65 ☐ NO GI symptoms ☐ Average colorectal cancer risk ☐ No prior colonoscopy

☐ Patient able to follow prep instructions

****EXCLUSION – REQUIRES CONSULT IF ANY BELOW APPLY.**

☐ Rectal bleeding ☐ Iron deficiency anemia ☐ Weight loss

☐ Persistent abdominal pain ☐ Change in bowel habits ☐ Positive FIT/Cologuard

☐ Personal history polyps or colorectal cancer/Family history of colorectal cancer/polyps

☐ IBD (Crohn's/Ulcerative Colitis) ☐ Prior colon resection ☐ Chronic diarrhea ☐ Severe constipation

MEDICAL HISTORY

Cardiac disease: ☐ No ☐ Yes- Describe_____

Pulmonary disease: ☐ No ☐ Yes- Describe_____

Vascular disease: ☐ No ☐ Yes- Describe_____

MEDICATIONS

Anticoagulants: ☐ None ☐ Aspirin ☐ Plavix ☐ Eliquis ☐ Xarelto ☐ Pradaxa ☐ Warfarin ☐ Other _____

GLP-1/SGLT-2: ☐ None ☐ Name: _____

Medication allergies: _____

BMI: _____

Medications: _____

FOR CENTER FOR DIGESTIVE HEALTH USE ONLY:

☐ Approved for Direct Colonoscopy-Mid-Level Review ☐ Requires Office Consultation

ClinLab
Clinical Testing Laboratory, Inc.

Clinical Testing Lab of California
55492 Spring Street, Suite 201
Vienna, CA 94078-9932
Phone: 510-555-1212

Specimen Number	Patient ID	Control Number	Account Number	Account Phone Number	Route

Patient Last Name	Account Address

Patient First Name	Patient Middle Name
Jeff	

Patient SS#	Patient Phone	Total Volume

Age (Y/M/D)	Date of Birth	Sex	Fasting
46 v.o.		Male	Yes

Patient Address	Additional Information

Date and Time Collected	Date Entered	Date and Time Reported	Physician Name	NPI	Physician ID

Tests Ordered
CBC, Platelet Ct, and Diff ; Liver Function (AST/ALT) ; Iron/Iron Binding Panel

General Comments

TESTS	RESULT	FLAG	UNITS	REFERENCE INTERVAL	LAB
CBC, Platelet Ct, and Diff					
Hematocrit	40.6		%	40.8 - 51.9	07
Hemoglobin	14.2		g/dL	14.6 - 17.8	07
Red Blood Cell Count	8.4		x10E6/uL	4.69 - 6.07	07
White Blood Cell Count	7.4		x10E3/uL	3.2 - 10.6	07
RDW	13.0		%	10.8 - 14.1	07
MCV	82.6		fL	77.8 - 94.0	07
MCH	27.4		pg	26.5 - 32.6	07
MCHC	34.4		g/dL	32.7 - 36.9	07
Imm.Granulocytes (Absolute)	6.2		x10E3uL	1.3 - 7.0	07
Granulocytes (Percent)	57.2		%	44.0 - 76.0	07
Eosinophils (Absolute)	0.3		x10E3uL	0.0 - 0.4	07
Eosinophils (Percent)	5.2		%	0.0 - 6.0	07
Basophil (Absolute)	0.08		x10E3uL	0.0 - 0.1	07
Basophil (Percent)	1.7		%	0 - 2	07
Monocytes (Absolute)	0.64		x10E3uL	0.2 - 0.7	07
Monocytes (Percent)	6.9		%	4 - 8.9	07
Lymphocytes (Absolute)	2.26		x10E3uL	0.8 - 3.1	07
Lymphocytes (Percent)	37.2		%	15 - 43	07
Platelets (Absolute)	388		x10E3uL	177 - 406	07
Liver Function					
Aspartate aminotransferase (AST)	223	High	U/L	8 - 43	07
Alanine Aminotransferase (ALT)	257	High	U/L	7 - 45	07
Iron and Total Iron Binding					
Serum Iron	2300	Very High	ug/dL	40 - 155	07
Total Iron Binding Capacity	328		ug/dL	250 - 400	07
Transferrin Saturation	72	Very High	%	20 - 50	07

PRELIMINARY INTERPRETATION:

A preliminary diagnosis of Hemochromatosis can be made based on very high Serum Iron and Transferrin Saturation values. Elevated liver enzyme levels are indicative of liver failure which is a common consequence of Hemochromatosis.

There are well established pathogenic genetic variants that are strongly correlated with Hereditary hemochromatosis. The presence of a hereditary disorder has consequences for family members. Genetic testing and counseling should be considered by the patient and family members.